

Section 10

Independent System Evaluations

Version 2.0 — Preliminary Draft

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SECTION 10 — INDEPENDENT EVALUATIONS AND EXTERNAL ASSESSMENTS

(Third-Party Evaluation Layer)

Version 2.3 — June 5, 2026

10.1 PURPOSE AND SCOPE

This section presents findings from independent evaluations commissioned by or presented to Iowa HHS / Iowa DHS by external evaluators. Unlike Section 9, which documents what Iowa HHS said about itself, Section 10 documents what independent organizations found when they examined Iowa's child welfare system. These external findings carry particular evidentiary weight because they cannot be dismissed as internal advocacy — they are the conclusions of third-party assessors brought in by the state itself to identify system failures.

The documents in this section span 2017–2023, creating a continuous independent audit record across the CFSR Round 3 monitoring period, the Round 3 PIP, and the transition into Round 4. All documents were either commissioned by Iowa DHS/HHS or presented to Iowa DHS/HHS leadership in an official capacity.

Cross-reference: Section 6 (CFSR longitudinal record); Section 9 (state-generated admissions); Section 7 (Round 4 structural integrity); Section 13 (Systemic Failure Matrix).

10.2 ATTACHMENT A18 — CWPPG TARGETED CHILD WELFARE REVIEW (2017)

Iowa Department of Human Services Initial Targeted Child Welfare Review

Conducted by: The Child Welfare Policy and Practice Group (CWPPG)

Date: October 31, 2017

Trigger: Two deaths of children in finalized subsidized adoptive placements

Source: <https://hhs.iowa.gov/media/9250/>

NOTE ON DOCUMENT CONTEXT: This review was commissioned following the deaths of two teenage girls in separate finalized adoptive homes who died of starvation. Both had been subjects of maltreatment reports following their adoptions that "did not result in intervention to prevent their deaths." This is the immediate precursor to the CFSR Round 3 onsite review conducted April-September 2018.

10.2.1 WORKFORCE AND WORKLOAD — CASELOAD FINDINGS

"Reviewers were told that case managers generally carry about 15 families or about 30 children in a combination of out-of-home care and in-home service cases. Assessment staff receive an average of 11.5 new cases per month."

"Actual caseloads in some counties were reported to be much higher than the statewide average. Reviewers were told this was true in both Polk and Linn counties. Some counties were reported to have ongoing caseloads as high as 40 families with child protection averaging over 20 cases per month."

(CWPPG, p.7)

AUDIT RELEVANCE: The CWPPG review documents that in 2017 — one year before the CFSR Round 3 onsite review — actual county-level caseloads were as high as 40 families per worker, far exceeding the statewide averages cited in Iowa HHS's own CAPTA report. This directly contradicts any narrative of adequate caseworker capacity during the case-level removal documented in Section 14.

10.2.2 STAFF MORALE — INDEPENDENT ASSESSMENT

"Staff morale in the service areas reviewers visited was described by many of those both within and outside of DHS as poor. Recent legislative changes in collective bargaining [contributed to this]."

(CWPPG, p.8-9)

10.2.3 DATA SYSTEM DEFICIENCY — 2017 ADMISSION

"The department's capacity to track and easily access and analyze data for ongoing services and children and families involved in out of home care is far less robust. Administrators report that it is badly outdated... Reviewers were told that DHS has outlined a plan for revision of the current child welfare system, but has no timeline for when resources may become available to build it."

(CWPPG, p.9)

AUDIT RELEVANCE: In 2017 — two years before the AECF assessment and six years before the CCWIS redesign documented in Section 9.3.1 — an independent reviewer found Iowa's child welfare data system was "badly outdated" with no timeline for replacement. The APSR FFY2026 (Section 9.3.1) documents that the CCWIS was still being redesigned in 2024-2025. This establishes a continuous, unresolved data system deficiency spanning at least eight years.

10.2.4 FAMILY ENGAGEMENT — INDEPENDENT FINDING

"Interviews with youth, parents and grandparents, foster parents, and DHS case managers indicate that many believe there is insufficient focus on engaging children's parents in assessing needs related to child safety, planning interventions to address them, and evaluating progress. One long-time external partner observed that the emphasis on working with families and on reunification seems to have been lost." (CWPPG, p.12)

AUDIT RELEVANCE: An independent reviewer documented in 2017 that "the emphasis on working with families and on reunification seems to have been lost." This is a third-party corroboration of the CFSR Round 3 findings that Iowa failed CFSR Item 2 (Services to Prevent Removal/Re-Entry) — the very item that Iowa's own APSR FFY2026 subsequently admitted was not achieved through the Round 3 PIP.

10.2.5 SUBSTANCE ABUSE SERVICE GAP — 2017 FINDING

"The most consistently cited area of need was in mental health treatment, especially insofar as in-patient services are concerned." (CWPPG, p.16)

"FSRP providers elsewhere in the state are reportedly offering some evidence-based intervention models including SafeCare, which is being offered by five of the eight FSRP providers." (CWPPG, p.16)

AUDIT RELEVANCE: The CWPPG identified in 2017 that mental health and in-patient services were the most consistently cited service gap — the same gap independently documented by the AECF in 2019 (Section 10.3) and still reflected in the CFSR Round 3 "no protocol" admission documented in Section 6.5.3.

10.2.6 FOSTER CARE ENTRY RATE — ABOVE NATIONAL MEDIAN

"Children enter and remain in out of home care in Iowa at a rate higher than the

national median." (CWPPG, p.14-15)

The quantitative data section documents:

- 2016: 18,481 child abuse assessments; 6,575 (36%) confirmed or founded
- 2017 (first 5 months): child abuse assessments increased 43% over prior year
- Reports accepted increased from 48% (2015) to 62% (first half 2017) of all calls

AUDIT RELEVANCE: Iowa's child abuse assessment volume was increasing substantially — 43% year-over-year in 2017 — with no corresponding increase in Social Worker III positions. This is the independent documentation of the structural capacity crisis that the CFSR Round 3 review would document six months later.

10.3 ATTACHMENT A19 — AECF PREVENTIVE SERVICES NEEDS ASSESSMENT (2019)

Iowa Preventive Services Needs Assessment: Summary of AECF Findings & Recommendations

Conducted by: The Annie E. Casey Foundation

Presented to: Iowa DHS

Date: June 18, 2019

Source: <https://hhs.iowa.gov/media/9253/>

10.3.1 DOCUMENT CONTEXT

The AECF Needs Assessment was presented to Iowa DHS on June 18, 2019 — exactly six months after the CFSR Round 3 PIP began (December 31, 2018). It is the result of a DHS/AECF partnership examining Iowa's preventive services system. The AECF is the nation's leading child welfare philanthropy organization and is frequently cited as an authoritative independent evaluator.

10.3.2 FOSTER CARE ENTRY RATE — ABOVE NATIONAL MEDIAN IN ALL AGE GROUPS

Slide 6 (p.6) presents AFCARS data showing Iowa's foster care entry rate per 1,000 children compared to the national median (FFY 2017):

Under 1 year old: Iowa 16.6 / National Median 13.6

1-5 years: Iowa 6.8 / National Median 4.4

6-9 years: Iowa 4.7 / National Median 3.1

10-12 years: Iowa 4.1 / National Median 2.6

13-15 years: Iowa 4.5 / National Median 3.1

16-17 years: Iowa 3.7 / National Median 2.6

"Iowa's foster care entry rate per 1,000 children is higher in every age group compared to the national median."

"From 2014-2018 there has been a 14.5% increase in entries."

"About 1/3 of screened in reports each year are for children who were previously substantiated."

(AECF, Slide 6, p.6)

AUDIT RELEVANCE: In June 2019, six months into the CFSR Round 3 PIP, an independent assessment by the nation's leading child welfare philanthropy found that Iowa's foster care entry rate exceeded the national median in every single age group, with entries increasing 14.5% over five years. This is the independent corroboration of the CFSR Round 3 findings that Iowa was failing its preventive services system.

10.3.3 SUBSTANCE ABUSE REMOVAL TREND — STATE'S OWN DATA

AECF slide 17 (p.17) presents Iowa AFCARS data:

Number of Children Removed for Parental Substance Abuse:

FFY 2014: 1,424

FFY 2015: 1,679

FFY 2016: 1,824

FFY 2017: 2,113

FFY 2018: 2,093

A 47% increase in substance-abuse-related removals between FFY 2014 and FFY 2018.

"Trends in feedback:

- There is a need to expand existing services (i.e. inpatient for parenting fathers)
- Inability to drug test is an issue
- Insurance/payment is preventing access
- Existing programs are full or too far away"

(AECF, Slide 17, p.17)

AUDIT RELEVANCE: The AECF documented in June 2019 that children removed for parental substance abuse increased 47% over five years, and that the service system to address this population had four documented access barriers: inability to drug test, insurance/payment barriers, programs at capacity, and geographic inaccessibility.

These are precisely the conditions under which the Iowa DHS drug testing policy limitations (A52, Section 9.3.8) operated and the service referral collapse documented in the CAPTA report (Section 9.4.2) occurred.

10.3.4 CASE MANAGEMENT SYSTEM FAILURES

AECF Slide 9 (p.9) documents "Prevailing challenges":

- "Duplicate responsibility between FSRP and DHS workers"
- "Disagreement about what FSRP workers should do to support families"
- "Limited time spent with families due to high caseloads, frequent turnover, and significant time spent traveling"

Quoted from a judge: "We had a family that had three different FSRP workers in three months. They can't develop effective working relationships in that amount of time."

(AECF, Slide 9, p.9)

AUDIT RELEVANCE: A judge quoted in the AECF's June 2019 assessment documents that FSRP turnover was so severe that families experienced three different workers in three months — identical to the CFSR Round 3 findings on caseworker continuity failures and the APSR FFY2026 admission that turnover increased training cohort sizes (Section 9.3.4).

10.3.5 PARENTING SERVICES SYSTEM FAILURE

"Many providers are using different models making it difficult for DHS to know what is provided or track effectiveness."

"Families are assigned randomly so there is no matching process."

"Almost half of all foster care entries in Iowa are children 0-5."

(AECF, Slide 14, p.14)

AUDIT RELEVANCE: With almost half of Iowa's foster care entries being children 0-5, the absence of a standardized parenting intervention model and random family assignment — documented by an independent evaluator in June 2019 — directly implicates the conditions documented in Section 9.10 (ECI Needs Assessment) and

Section 6.5.3 (Round 3 "no protocol" for drug-removal population).

10.3.6 AECF CORE RECOMMENDATION — "RE-ENVISION THE CONTINUUM"

"Iowa has an opportunity to re-envision its continuum of prevention services to better meet the needs of children and families."

"Iowa can achieve improved results in prevention services by:

- Developing a clear case management model
- Using targeted evidence-based interventions
- Instituting stronger accountability for DHS and Providers"

(AECF, Slide 2, p.2)

AUDIT RELEVANCE: The AECF's core recommendation in June 2019 was that Iowa needs to develop a clear case management model — six months into the Round 3 PIP, after three CFSR cycles. This is the independent documentation that Iowa entered its fourth federal review cycle without a clear case management model in place.

10.4 ATTACHMENT A20 — CHANGE & INNOVATION AGENCY FINAL REPORT (2023)

Iowa Department of Health and Human Services Child Welfare Final Report of Findings and Recommendations

Conducted by: Change & Innovation Agency (C!A)

Date: 2023 (commissioned post-Round 3 PIP completion)

Scope: Workforce/Workload, Policy/Practice Review, Quality/Accountability, Technology/Data Integration

Source: <https://hhs.iowa.gov/media/11516/>

10.4.1 DOCUMENT CONTEXT

The C!A Report is the most comprehensive independent systems assessment of Iowa child welfare in the CFSR Round 4 era. It covers 217 pages and examines all major functional areas. It was commissioned by Iowa HHS following the December 31, 2023 completion of the Round 3 PIP — making it both a retrospective evaluation of what the PIP achieved and a baseline for Round 4. Its central question mirrors the GAGAS audit standard: "Are children and families better off because of HHS intervention?"

10.4.2 STRUCTURAL ISSUES — EIGHT IDENTIFIED SYSTEM deficiency categories

The C!A methodology section (p.13) explicitly identifies eight structural issues that "may contribute to poor outcomes for families involved in the child protection system":

1. Underfunding and Resource Shortages: "Insufficient funding and resources for child welfare agencies can lead to overworked staff, high caseloads, and limited access to essential services for families. This can hinder timely and effective interventions."
2. Lack of Coordination: "Fragmented or poorly coordinated services across agencies (e.g., child protection, mental health, substance abuse treatment, juvenile justice) can result in families not receiving comprehensive and integrated support."
3. Caseworker Turnover: "High turnover rates among child protection caseworkers can disrupt continuity of care and relationships with families, making it challenging to provide consistent support and services."

4. Inadequate Training and Support: "Caseworkers may not receive sufficient training or ongoing supervision to effectively assess and address complex family dynamics, leading to misjudgments and potentially harmful decisions."

5. Racial and Cultural Disparities: "Structural racism and cultural insensitivity within child welfare systems can result in disproportionate interventions and poor outcomes for minority families due to bias and discrimination."

6. Overemphasis on Removal: "Some systems may prioritize child removal over family preservation due to policies or a lack of resources for in-home support services, potentially leading to unnecessary family separations."

7. Lack of Preventative Services: "Limited access to preventative services, such as parenting education, mental health support, and substance abuse treatment, can result in families reaching crisis points before receiving assistance."

8. Legal and Procedural Delays: "Lengthy legal processes can delay permanency planning and reunification efforts."

(C!A, p.13)

AUDIT RELEVANCE: The C!A Report's own methodology framework documents eight structural deficiency categories — and then finds evidence of Iowa's child welfare system demonstrating these exact structural failures. This is the independent cataloguing of Iowa's system against a nationally-recognized structural failure taxonomy, conducted after the completion of the Round 3 PIP.

10.4.3 WORKFORCE TURNOVER — QUANTIFIED BY SERVICE AREA

The C!A Report documents 2022 staff attrition rates by service area (p.21):

Attrition Rates — 2022 by Service Area:

Des Moines (DMSA): SW2: 44% / SW3: 34% overall

Northern (NISA): SW2: 44% / 37 positions vacated

Eastern (EISA): SW3: 39% / 28% overall

Cedar Rapids (CRSA): SW3: 30% overall

Western (WISA): SW2: 22%

All Totals: SW3: 35% / 135 exits; SW2: 26% / 202 exits

"Northern service area... increase of number of exits from 9 to 37 vacated positions in 2022 compared to 2020." [311% increase in exits in two years]

"DMSA and NISA... reported 44 percent turnover in SW2 staff in 2022."

(C!A, p.21)

AUDIT RELEVANCE: Post-PIP completion, Iowa's child welfare workforce was experiencing 44% annual turnover in the state's two largest service areas. The APSR FFY2026 (Section 9.3.4) documents this same trend continuing through 2023-2024, confirming the C!A finding was not anomalous but structural.

10.4.4 OVERTIME AS SYSTEM SURVIVAL MECHANISM

"Without overtime, the system would breakdown and there would be no way for the workforce to keep up with the workload."

"For the last three years the SW2 and SW3 OT expenditures have hovered between \$1.6 and \$1.8 million. Des Moines and Western service areas consistently account for 40-50 percent of annual OT expenditures."

(C!A, p.22)

AUDIT RELEVANCE: The C!A Report independently documents that Iowa's child welfare system has been structurally dependent on overtime for at least three years to avoid operational collapse. This is the independent corroboration of the APSR admission that Iowa "does not currently set a maximum caseload" (CAPTA, p.34). A system that requires \$1.6-1.8 million in annual overtime to function — and would "breakdown" without it — is not a system operating at adequate capacity.

10.4.5 WORKLOAD INCREASE — 14%/8%/6% DOCUMENTED

"Growing workloads (14 percent more calls, 8 percent more allegations, and 6 percent increase in time in care) have local offices concerned that demand is, or has already, surpassed their ability to keep up."

(C!A, p.19)

"Their optimism is challenged by the reality of the workload and many report feeling as if the current level of performance is beginning to diminish."

(C!A, p.19)

10.4.6 ASSESSMENT TURNOVER — 25% OVERALL; 30%+ FOR CASE MANAGEMENT

"While Assessment staff report 25 percent turnover overall, there is much variation depending on the location of the office. Offices in proximity to larger population centers (Polk, Ames, Pottawattamie) reported a higher level of turnover and recently hired Social Worker III (SW3) staff with no experience in child welfare."

"Ongoing Case Management was reported as having the highest percent of annual turnover (+30 percent) and the highest number of staff with less than one year of

experience (ninety-six)."

(C!A, p.20)

AUDIT RELEVANCE: The C!A Report documents that as of 2023, Iowa's ongoing Case Management function — the function responsible for monitoring child safety in foster care and in-home cases — had the highest turnover rate in the system (30%+) and 96 workers with less than one year of experience. This is the independent quantification of the "post-permanency blind spot" documented in Section 7.4.

10.4.7 INTAKE AND ASSESSMENT INCONSISTENCIES

"Almost universally Assessment workers reported perceived inconsistencies in what constitutes a screen in versus a reject, categorization as a child abuse assessment versus a family assessment, and the response timeframes."

(C!A, p.19)

AUDIT RELEVANCE: In 2023 — after completing a Round 3 PIP specifically targeting safety assessment improvements — independent evaluators found that assessment workers "almost universally" reported inconsistencies in fundamental screening decisions. This directly contradicts any claim that the Round 3 PIP resolved the safety assessment failures documented in Section 6.5.

10.4.5 DENIAL OF CRITICAL CARE — POVERTY AND SUBSTANCE ABUSE AS PRIMARY DRIVERS

"In Iowa the highest volume category of abuse is Denial of Critical Care (neglect). Nearly all of these cases have substance abuse and/or domestic violence as the reported concern." (C!A, p.102)

AUDIT RELEVANCE: The C!A independently confirms what the CFSR Rounds 1-3 documented and Section 7 establishes: Iowa's child welfare system is dominated by substance-abuse-related neglect cases (DCC), yet the service infrastructure for addressing substance use disorder was the most consistently cited gap across every independent evaluation from 2017 through 2023.

10.4.6 ASSESSMENT VOLUME AND CAPACITY — KEY DATA FINDINGS

The C!A Report documents Iowa's child welfare system data from 2021 (p.40-42):

System Flow (per 100 intake contacts):

- 64.5% accepted for assessment (42,556 total assessments)
- 30% confirmed or founded findings

- 13% of all contacts resulted in confirmed/founded findings
- 8 per 100 contacts resulted in in-home services (avg. 8.7 months)
- 3 per 100 contacts resulted in removal of at least one child
- 2 per 100 children expected to reunify after 1.9 years
- 1 per 100 children expected to be adopted after 2.9 years

Key capacity findings (p.41-42):

"Assessment staff in Iowa are assigned approximately 43,000 family assessments and child abuse assessments from Intake annually." (p.31)

"SW2 staff experienced a 57 percent increase in turnover from 2019 to 2021." (p.34)

"The average number of days in foster care increased by 6 percent over the past 4 years."

"On average, a child spends 30.8 months in foster care, out-of-home case closure occurs at 22 months, and in-home cases are closed at 8.8 months."

"There is approximately a ten-month difference between TPR and adoption finalization."

"Children reunifying are spending almost two years in care. (23.7 months)"

"Adoptions are taking almost 3 years to complete (34.9 months)"

(C!A, p.42)

"Report volume is increasing, and 2021 data shows a 9.6 percent increase over the 2018/2019 average."

(C!A, p.41)

AUDIT RELEVANCE: The C!A's system flow data documents that children in Iowa who enter foster care spend an average of 30.8 months in out-of-home care, with children reunifying spending nearly two years in care. TPR to adoption finalization averages an additional ten months. These are the timelines operating in the same system that the CFSR documented failing its permanency outcomes across three rounds.

10.4.7 CONCURRENT PLANNING — DOCUMENTED FAILURE

"SW2s and supervisors reported that concurrent planning is happening late if it is happening at all. SW2s reported they generally know the direction of the family somewhere between three to six months, but the Adoption workers typically do not get involved until after the first year, or even later. The state's practice is to have concurrent planning begin sixty days after removal from the home. SW2s reported

this is not consistently happening during that time period."

(C!A, p.36)

"Adoption workers reported that efforts to locate parents, genograms, and ecomaps are seldom exhaustive due to the capacity issues of SW2s. Workers believe this could be because of the delay in family engagement during the first sixty days of a case, SW2s not being comfortable or familiar with concurrent planning questions, and many workers simply giving 'a packet to the family instead of doing an interview with them.'"

(C!A, p.37)

"Case Management workers are not getting the supervision they need, so when things are not getting done by the time termination occurs, it's up to the Adoption worker to complete the work." (C!A, p.36)

AUDIT RELEVANCE: The C!A documents that Iowa's concurrent planning requirement — begin within 60 days of removal — was not being consistently met. This is the direct operational failure underlying the CFSR Round 3 findings on permanency outcomes. Case managers were completing concurrent planning late or not at all due to capacity constraints, while adoption workers were inheriting incomplete case preparation at TPR.

10.4.8 CASE TRANSFER GAPS — FAMILIES LEFT WITHOUT SERVICES

"Case Managers reported significant challenges with the case transfer process which is consistently not resulting in a warm handoff and often results in delays in access to services and general confusion of the family. Workers reported families sitting in limbo for weeks without access to services and inadequate visitation with their children."

(C!A, p.114)

"There is no formally communicated internal protocols for how SW2s and SW3s work together... The system appears to be almost entirely driven by timelines versus quality and complete work. The case transfer practices from Assessment to Case Management often result in a gap in services to and visits with families." (C!A, p.51)

AUDIT RELEVANCE: The C!A documents that families routinely experienced gaps in services and visitation at case transfer — the exact transition point where the case-level record documented failures in Section 14. This is the system-level explanation for case-level service gaps that were documented in the Round 3 onsite review period.

10.4.9 SERVICE ARRAY — MENTAL HEALTH GAP UNIVERSALLY REPORTED

"In many counties across the state, there are limited community-based services to address the underlying poverty-related needs of families. In addition, there are gaps

for mental health services for youth and adults—both with and without child welfare involvement. These gaps significantly limit the ability of SW3s to connect families to needed services early on and potentially mitigate their need for formal involvement and case oversight." (C!A, p.52)

"Workers who were not qualified to provide a high-quality service to families."

"Provider staff not meeting with families in a timely manner."

"The current contract does not appear to be fulfilling requirements nor meeting the needs of families, especially when it comes to solution-based casework. Despite updates in contract language, prescribing specific tasks and timelines, best practices are not routinely being achieved." (C!A, p.52-53)

AUDIT RELEVANCE: The C!A independently documents — after completion of the CFSR Round 3 PIP — that contracted FCS providers were not meeting contract requirements for service delivery and that mental health service gaps existed statewide. This corroborates the CWPPG 2017 finding (Section 10.2.5), the AECF 2019 finding (Section 10.3.6), and the Provider Forums 2019 finding (Section 10.5.3) on the same gaps — documented across six years without resolution.

10.4.10 TPR DRIVEN BY COURTS AND COUNTY ATTORNEYS, NOT FAMILY PROGRESS

"Confusion exists related to TPR timelines/criteria, and staff reflected that TPR decisions are sometimes driven by the court and county attorneys rather than based on recommendations from HHS." (C!A, p.48)

"The push for children to enter the system in a voluntary status can be linked to the perceived level of control the courts possess over cases once involved... in many areas of the state courts are setting the direction and pace of cases." (C!A, p.35)

"These other documents and court reports do not set conditions to return home, resulting in inconsistent messaging to the family about what they need to do to get their children back and get out of the system." (C!A, p.35)

"Significant turnover among judges (reported at 60 percent) and SW2s (35 percent) are also resulting in additional challenges with court and state alignment." (C!A, p.53)

AUDIT RELEVANCE: The C!A independently documents that TPR decisions in Iowa are driven by court and county attorney timelines rather than by family progress assessments — the system-level mechanism underlying the case-level record in Section 14. Court-driven TPR timelines that are set at the first hearing (corroborated by Provider Forum testimony in Section 10.5.3) are the independent documentation

of the predetermined TPR framework that operates regardless of family trajectory.

10.4.11 TRAINING FAILURES — NEW WORKERS TRAINING NEW WORKERS

"Staff shared they are/were not given adequate time to train, shadow, and be mentored before getting a caseload. Staff indicated some new workers resigned due to being overwhelmed by their caseload and lack of training. In many service areas, new SWCMs are training new SWCMs." (C!A, p.53)

"Supervisors do not typically go out on cases with new staff, rarely join them in court..." (C!A, p.53)

AUDIT RELEVANCE: "New SWCMs are training new SWCMs" — this is the operational description of the workforce cascade: the 30%+ annual turnover (Section 10.4.3) produced a workforce where the most experienced person available to train a new worker was often another new worker. This directly explains the CFSR Round 3 finding of inadequate caseworker skill and the C!A's own finding of 96 case managers with less than one year of experience.

10.4.12 KINSHIP — PLACEMENT SHORTAGES AND FINANCIAL BARRIER

"HHS received a kinship navigator grant, so when a child is placed with relatives or suitable others, HHS makes a referral to a kinship navigator... However, the kinship navigators are a new program that is not running as efficiently as it ultimately might. Assessment workers are placing most removed children in relative or kinship care and finding that many may not get out to the home to assist the family in a timely manner."

"Kinship families are eligible for six months of caretaker financial assistance, set at \$10 per day, and are encouraged to become licensed. The kinship financial assistance is limited to 6 months. If the family is still not licensed, they may apply for FIP (Iowa's TANF cash assistance) however, no other concrete support is provided." (C!A, p.38)

"The lack of availability of licensed placements was a significant and repeatedly stated concern among supervisors and staff across the state. Staff reported 'not expecting good results' when they must contact the contracted provider to secure foster placement." (C!A, p.38)

AUDIT RELEVANCE: Iowa's kinship support payment was \$10/day — \$70/week. The Harkin Institute analysis (Section 10.6.2) documents the typical CCA family income at \$18,287/year. A kinship caregiver adding a child to their household received \$10/day for 180 days (\$1,800 total) — a structurally inadequate support for kinship placement stability. The C!A documents the predictable result: kinship placements were

unstable, licensing was delayed, and placement options were scarce.

10.4.13 DISPROPORTIONALITY — UNRECOGNIZED BY FRONTLINE STAFF

"When asked about whether and to what extent inequities were showing up in Iowa, workers identified poverty as the primary driver and not race or ethnicity. Staff who identified disproportionality as an issue noted it in the context of reporters, particularly school officials. There were also comments made about the need to 'educate' immigrant families on 'how we do things in America,' suggesting a lack of cultural humility, at minimum. Workers and supervisors were consistently unable to articulate where and to what extent disparities or disproportionality exist in the state." (C!A, p.50)

AUDIT RELEVANCE: The C!A independently documents that frontline Iowa HHS staff could not articulate disproportionality patterns in their own caseloads — and that some staff comments evidenced a lack of cultural humility. This is a structural accountability gap: if workers cannot identify racial and ethnic disparities in their case decisions, no individual-level accountability mechanism can correct them.

10.4.14 TECHNOLOGY — FIVE DOCUMENTED CCWIS deficiency categories

The C!A Technology section (p.54-56) documents five categories of documented CCWIS failures as reported by frontline staff:

1. System does not integrate information between teams, programs, and systems. Staff "must access several additional systems to effectively do their jobs."
2. System requires duplicative data entry. Staff requested: "Eliminating duplicate data entry."
3. Document management is "cumbersome and ineffective." Case files are "incredibly large... where the right document cannot be found."
4. System data is "not updated in a timely manner resulting in a lack of trust." Supervisors and staff keep their own spreadsheets to track actual case status.
5. System "reliability" failures: "Almost all teams expressed frustration with the frequency of planned and unplanned system downtime."

AUDIT RELEVANCE: The C!A Technology section provides the independent frontline worker documentation of the CCWIS failures documented at the administrative level in Section 9.3 (IAPD, Google/Carahsoft contract). Staff were keeping their own spreadsheets because the state's child welfare data system was unreliable. This is the operational consequence of the \$41.9M CCWIS contract documented in Section 9.

10.4.8 C!A CORE RECOMMENDATIONS — SYSTEMIC GAPS

The C!A Organizational Wide recommendation (p.9) calls for:

- "Transparently using data and QA practices for decision-making and CQI"
- "Maintain appropriate staffing levels to meet the goals of the organization"
- "Expansion of the service array to provide timely/immediate access to appropriate services"
- "Effective contract management practices, processes, and procedures"

AUDIT RELEVANCE: Post-PIP, the C!A's top recommendations are that Iowa needs to: maintain appropriate staffing (acknowledging it currently does not), expand its service array (acknowledging current gaps), and use data transparently (acknowledging current data practice deficiencies). These are the same foundational gaps documented across three CFSR cycles.

10.5 ATTACHMENT A21 — CWPPG SERVICE PROVIDER FORUMS REPORT (2019)

Iowa Department of Human Services Provider Forums — Voice and Vision

Conducted by: The Child Welfare Policy and Practice Group (CWG)

Date: June 11, 2019 (forums conducted February-March 2019)

Participants: ~110 FSRP Care Coordinators, supervisors, provider directors, CASA representatives

Methodology: 10 statewide forums; no DHS employees present; participants assured confidentiality

Locations: Bettendorf, Council Bluffs, Des Moines, Iowa City, Johnston, Mason City, Osceola, Sioux City, Washington, and Waterloo

Source: <https://hhs.iowa.gov/media/9250/>

10.5.1 DOCUMENT CONTEXT

This report documents the views of Iowa's child welfare service provider community at the beginning of the CFSR Round 3 PIP period (December 2018 – December 2023). It is unique in the evidentiary record because it captures the perspectives of practitioners who work directly with families — without DHS employees present — giving it particular credibility as an unfiltered assessment of systemic conditions. The CWG facilitators explicitly noted that providers had given similar input before and doubted it would be used, stating at the conclusion: "Please don't let these meetings be the end."

10.5.2 DRUG TESTING — CONCURRENT INDEPENDENT FINDING

The Provider Forums report documents provider concerns about drug testing policy that directly parallel the case-level record in Section 14 and the Iowa DHS drug testing policy document (A52, Section 9.3.8):

"Several asserted that too many parents are 'punished by removals' more for doing something illegal than for presenting true risk to the children, citing that positive drug screens are frequently reason for removals or barriers to reunification, even if no safety concerns are noted." (CWPPG Forums, p.20)

"Providers consistently maintained that 'too much weight' is given to positive drug screens as plans for permanency are established and monitored." (p.21)

"Families with long-term sobriety or other wellness issues typically need more than six months to demonstrate progress." (p.21)

The CWPPG facilitators document that Iowa's own DHS website language already stated in 2019: "Due to its limitations, drug testing results should not be relied on as the sole measure in determining issues of safety and risk." (p.20)

And that the Iowa Children's Justice State Council's 2011 bench note guidelines listed "inappropriate uses for drug and alcohol testing" including: "when used as the sole indicator of a parent's progress... or when used as punishment to a parent." (p.21)

AUDIT RELEVANCE: In February-March 2019 — concurrent with the beginning of the Round 3 PIP — 110 Iowa child welfare providers documented that drug testing was being used as the sole basis for removals and barriers to reunification despite the state's own published guidance prohibiting this practice. This is a direct third-party corroboration of the misuse of drug testing in the case-level record documented in Section 14, and the post-hoc admission in Iowa HHS Communication 626 (A52) that drug testing alone cannot be the basis for TPR.

10.5.3 TPR PROCESS — PROVIDER TESTIMONY

"At the very first hearing, provider was told that DHS would be 'terminating parental rights' in six months, so regardless of what the family accomplished through services, she had already been informed that TPR would occur. She asserted that this was unfair to everyone involved and was a demonstration of DHS struggling to meet TPR standards as opposed to assessing family progress." (CWPPG Forums, p.19)

"Another participant mentioned that a DHS Case Manager came to the removal hearing of an infant and immediately stated 'we are going for TPR in six months'." (p.19)

AUDIT RELEVANCE: Provider testimony at 10 statewide forums in February-March 2019 documents that DHS was announcing TPR timelines at first hearings — before families had any opportunity to demonstrate progress — as a systemic practice. This is a third-party corroboration of the framing that TPR was predetermined in the case-level record documented in Section 14, rather than reflecting a genuine assessment of family progress.

10.5.4 COMMUNICATION AND ROLE CONFUSION — #1 CONCERN

"More than any other question or prompter on the agenda, including funding, the issues of role clarification, relationships, and communication were discussed with exceeding frustration. An essential message from providers was that families were not well-served because too many 'don't know who's driving'." (CWPPG Forums, p.17)

"One participant said 'all we really share right now is mutual turmoil'." (p.18)

"At one forum, a Care Coordinator said she and others wanted to pose this question to Case Managers: 'Why do you hate us?'" (p.18)

"One supervisor, when asked about any need for clarity, said 'I just tell my staff to be the case manager'. Another supervisor added 'We are colliding with each other and creating an us vs. them environment.'" (p.18)

The CWPPG Report quotes Iowa's own 2018 APSR directly to corroborate these findings:

"A lack of clarity on roles and responsibilities was noted, as some stakeholders indicated that contract child welfare workers handle case management whereas the statewide assessment identifies DHS staff only as having case management responsibility. Turnover is high, particularly among contract providers, especially in urban areas of the state, leading to inconsistency for children and families that negatively affects their access to services." (2018 APSR, quoted in CWPPG Forums, p.17, 19)

AUDIT RELEVANCE: The 2018 APSR's own admission about role confusion and turnover is independently corroborated by 110 providers at statewide forums. The same APSR language appears in both documents — establishing that Iowa DHS acknowledged the role confusion in its own federal compliance report while it was simultaneously occurring in the case-level record.

10.5.5 TOP THREE REASONS FOR FOSTER CARE ENTRY WITH SERVICES IN PLACE

Forum Question 11 asked providers directly: what are the top three reasons children enter out-of-home care when preventive services are in place?

- "1. Lack of continuum of services for parental substance use disorder — severe lack in rural areas
2. Domestic violence not impacted by services or services not readily available
3. Untreated behavioral health needs of parents"

(CWPPG Forums, p.14)

AUDIT RELEVANCE: Iowa's service providers — across the state, in 2019 — identified parental substance use disorder service gaps as the primary reason children entered foster care even with preventive services in place. This is the frontline-worker corroboration of the

AECF's finding (Section 10.3.3) and the CFSR Round 3 "no protocol" admission (Section 6.5.3).

10.5.6 TOP THREE CHALLENGES FOR FAMILIES NOT MEETING IN-HOME GOALS

Forum Question 12 asked: top 3 challenges for families that aren't able to meet preventive services goals within 12 months?

- "1. Unsuccessful substance use disorder treatment
2. Transportation
3. Lack of natural supports, including respite assistance"

(CWPPG Forums, p.14)

AUDIT RELEVANCE: Iowa providers identify failed SUD treatment as the primary reason families don't achieve in-home case goals within 12 months — the same service gap documented by every external evaluator from 2017 through 2023, and the same gap that the AECF (2019) documented with a 47% increase in substance-abuse removals over five years.

10.5.7 KINSHIP UNDERUTILIZATION — PROVIDER TESTIMONY

"Providers perceived an underutilization of kinship care; one noted that a particular supervisor in one service area 'didn't like it and didn't know how to do it, so she didn't.'" (CWPPG Forums, p.19)

AUDIT RELEVANCE: Provider testimony documents that kinship placement decisions in Iowa were dependent on individual supervisor preferences rather than policy-driven practice — the independent corroboration of the kinship exclusion documented in the case-level record (Section 14) and the APSR case review data showing only 43% kinship placement rate (Section 9.2.2).

10.5.8 EVIDENCE-BASED PRACTICES — FIDELITY DESTROYED BY DHS MODIFICATIONS

"The consensus was that EBP's are changed by DHS and then become 'unrecognizable as the EBP we submitted'. Participants were concerned that one serious impact on families has been that cases remained open for longer periods of time since the EBP had been significantly changed from original submission." (CWPPG Forums, p.2)

"Some voiced concern that the current EBP (The North Carolina Family Assessment Scale) is required to be completed monthly, yet it does not seem to be used to guide the case planning and casework process." (p.10)

AUDIT RELEVANCE: Provider testimony documents that Iowa DHS systematically modified EBPs to the point of being "unrecognizable" — the independent corroboration of the AECF finding (Section 10.3.5) that "families are assigned randomly so there is no matching process."

Both documents establish that Iowa's evidence-based practice infrastructure was compromised at the point of delivery in the CFSR Round 3 PIP period.

10.5.9 SALARY/PERFORMANCE INCENTIVE STAGNATION — 2007 TO 2019

The CWPPG Forums document documents that performance-based incentive payments for FSRP providers had barely increased in 12 years:

"a. Children are safe from abuse... (\$105 per case). This figure represents a \$5.00 increase from \$100.00 in 2007.

b. Children are safely maintained in their own homes... (\$263 per case). This figure represents a \$13.00 increase from \$250.00 in 2007.

c. Children are reunified within 12 months... (\$525 for each child). This figure represents a \$25.00 increase from \$500.00 in 2007."

(CWPPG Forums, p.6)

Provider-recommended salary for a Care Coordinator with a Bachelor's degree: \$43,000

(CWPPG Forums, p.8)

"Providers noted that the most recent increase in funds to providers was in 2015." (p.23)

AUDIT RELEVANCE: Performance incentives for preventing child removal had increased by \$5-25 per case over 12 years (2007-2019). The base payment was \$708/month per family. This is the independent documentation of the chronic underfunding of the contractual service infrastructure that Iowa's child welfare system depends on to demonstrate "reasonable efforts."

10.5.10 PROVIDER SKEPTICISM — "NOTHING HAS CHANGED"

"One attendee said she believed she was speaking for the provider community as a whole when she expressed great disappointment that 'nothing has changed' with funding or requests to the legislature, in spite of recommendations made in the recent past by The Coalition for Family and Children's Services in Iowa, Alliance, and CWG." (CWPPG Forums, p.28)

"Although they appreciated the opportunity, they had given input before and did not think it had been valued or utilized in planning for funding, service development, or relationship-building, and they left the forums with the palpable air of 'we'll see'." (p.27)

AUDIT RELEVANCE: At the start of the CFSR Round 3 PIP (2019), Iowa's frontline service providers documented — in a neutral, confidential forum — that prior rounds of input and recommendation had produced no meaningful change. This is the independent documentation of the "PIP accountability inversion" pattern established in Section 6.6: improvement processes are initiated, input is collected, and structural conditions remain unchanged.

10.6 ATTACHMENT A22 — HARKIN INSTITUTE KINDERTRACK ANALYSIS (2020)

Analysis of Iowa's Child Care Assistance Database — Executive Summary

Authors: Shelley L. Horak, EdD(c), MPH, CHES, Research Fellow, The Harkin Institute for Public Policy & Citizen Engagement; Caitlin E. Ward, MS, PhD Candidate, Department of Biostatistics, University of Iowa

In partnership with: Iowa Department of Human Services Bureau of Child Care Services

Date: February 5, 2020

Data source: Iowa DHS KinderTrack database (data pulled November 8, 2018; records from July 1, 2010)

Source: <https://harkininstitute.drake.edu/wp-content/uploads/sites/103/2021/03/02-01-2021-Kindertrack-Cover-Page.vFINAL.pdf>

NOTE ON EVIDENTIARY SCOPE: The Harkin Institute KinderTrack analysis examines Iowa's Child Care Assistance (CCA) program — the federally-funded childcare subsidy system — not the child welfare case management system directly. Its relevance to this petition operates through two channels: (1) it documents the economic and structural conditions of the families that the child welfare system serves; and (2) it documents the childcare infrastructure gaps that "reasonable efforts" referrals depend upon. The CCA population is the same population documented in the 2019 ECI Needs Assessment (Section 9.10) and the CFSR record as the primary child welfare-involved population.

10.6.1 CCA POPULATION PROFILE — WHO THE CHILD WELFARE SYSTEM ACTUALLY SERVES

"We found that almost 80% of families using CCA in Iowa are single-parent families, and most single-parents are single mothers, who account for 64% of all families enrolled in Iowa's CCA program."

Family Structure of Iowa CCA Program (Table 1):

- Single-parent family: 102,018 families (76.69%)
- Single mothers: 85,691 families (64.43%)
- Single fathers: 10,429 families (7.84%)
- Two-parent family: 26,406 families (19.86%)
- Relative family: 4,535 families (3.41%)

(Harkin Institute, p.18)

AUDIT RELEVANCE: The families enrolled in Iowa's Child Care Assistance program — 64% single mothers — are structurally identical to the population enrolled in Iowa's child welfare system. The ECI Needs Assessment (Section 9.10) documents 49% of Iowa's children are born into poverty; the ICAPP report (Section 9.8.3) documents 52% of ICAPP families had annual incomes of \$20,000 or less. This Harkin Institute

analysis quantifies the specific household structure and income profile of the population that Iowa DHS's "reasonable efforts" service referrals are directed at.

10.6.2 FAMILY INCOMES — MEDIAN AT \$15,387–\$18,287 FOR TWO-PERSON HOUSEHOLDS

Median annual income for CCA families, two-person household (Table 4):

2010: \$15,387

2015: \$17,490

2018: \$18,287

(Harkin Institute, p.22)

CCA eligibility threshold: 145% of federal poverty level = \$30,929 for a family of three in 2019. The median income for a two-person CCA household in 2018 was \$18,287 — approximately 59% of the poverty threshold for a family of three.

The "cliff effect" documented in the Background section (p.6-7): families lose \$5,000–\$9,000 per year in child care benefits when their income crosses the 145% FPL threshold — creating a direct disincentive to income advancement.

AUDIT RELEVANCE: The family income data establishes that Iowa's CCA/child welfare population is operating at approximately 59% of the poverty threshold — in a state where the average annual cost of infant childcare is \$10,131 (documented in Section

9.10.8, citing the same ECI 2019 data). For these families, child care cost represents 55% of annual household income. This is the financial context in which Iowa DHS made "reasonable efforts" determinations and referred families to services.

10.6.3 PROTECTIVE SERVICES — STEADILY INCREASING CCA NEED CATEGORY

"We observe that after mid-2012, the second most common reason reported for needing child care assistance was protective services, which is steadily increasing to just over 5% of parents by the end of 2018." (Harkin Institute, p.23)

The "Need for the Program" analysis tracked parent-reported reasons for CCA participation from January 2012 to December 2018:

- Employment: ~75% (2012) increasing to ~85%+ (2018) — dominant reason
- Protective Services: steadily increasing to 5%+ of parents by end of 2018
- Education: declining from 12% (2012) to 1% (2018)
- PROMISE JOBS: declining from 6.5% (2012) to ~2% (2018)

(Harkin Institute, p.23)

"Single mothers were also more likely to need child care due to protective services."

(Harkin Institute, p.24)

AUDIT RELEVANCE: The KinderTrack analysis documents a steady increase in CCA participation driven by child welfare protective services involvement between 2012 and 2018 — the CFSR Round 2 PIP period through the beginning of the Round 3 PIP. This is the independent database-level confirmation that Iowa's child welfare system was generating an increasing demand for CCA services at the same time that the ECI 2019 Needs Assessment documented a 42% collapse in CCA providers.

10.6.4 PROVIDER NETWORK COLLAPSE — DATABASE EVIDENCE

The KinderTrack provider count data (Table 6) shows the total number of providers participating in Iowa's CCA program declined significantly over eight years:

July 2010: 9,135 total providers

July 2011: 10,067 total providers

July 2012: 10,229 total providers (peak)

July 2013: 9,416 total providers

July 2014: 8,346 total providers

July 2015: 7,735 total providers

July 2016: 7,600 total providers

July 2017: 6,423 total providers

July 2018: 5,501 total providers

(Harkin Institute, Table 6, p.37-38)

That represents a 46% decline in CCA providers from peak (10,229) to 2018 (5,501).

"The total amount paid to non-registered child care homes has decreased."

(Harkin Institute, p.30)

AUDIT RELEVANCE: The KinderTrack database independently documents a 46% collapse in the provider network between 2012 and 2018 — corroborating the ECI 2019 Needs Assessment finding (Section 9.10.8) of a 42% drop in total child care programs since 2013. Both analyses document the same underlying trend from two different data sources. When Iowa DHS made "reasonable efforts" referrals to child care services during this period, it was referring families into a provider network that had lost nearly half its capacity.

10.6.5 REIMBURSEMENT BELOW MARKET RATE — PROVIDER PAYMENT GAP

"Payments to all types of providers have been less than the market rate, however the difference between market rate and amount paid is most pronounced for licensed

centers and registered child development homes."

"Biweekly amounts paid to providers are slightly below market rate, with the median amount paid per child at \$166 and median market rate at \$184."

"The median annual payments per child (family) about \$90 (\$160) lower than market rate in 2010 and \$220 (\$480) lower in 2018." (Harkin Institute, p.30-31, 41-42)

AUDIT RELEVANCE: Iowa's CCA reimbursement to providers was consistently below market rate and the gap was widening over time — explaining why providers were exiting the CCA network. The KinderTrack analysis establishes the financial mechanism that drove the 46% provider network collapse: being reimbursed \$220/year below market rate per child (2018) meant providers could not sustain their operations through CCA participation alone. This is the independent economic explanation for the childcare infrastructure collapse documented in the ECI 2019 Needs Assessment.

10.6.6 CLIFF EFFECT — STRUCTURAL ECONOMIC TRAP

"The cliff effect is a phenomenon that occurs when families lose their child care benefits due to an increase in income that exceeds the criteria for the program, while that increase in income is not enough to cover the cost of child care that had been funded through the CCA program. Many families in this situation turn away from raises and opportunities for advancement to avoid the loss of assistance: \$5,000 to \$9,000 per year." (Harkin Institute, p.7)

AUDIT RELEVANCE: The Harkin Institute documents a structural economic trap — the cliff effect — built into Iowa's child care assistance architecture. Families receiving "services" through CCA-connected child welfare referrals are operating in a system designed with an income cliff that actively disincentivizes the economic advancement that would theoretically reduce child welfare involvement. This is the independent documentation of the structural economic contradiction in Iowa's simultaneous operation of CCA and child welfare programs as coordinated "reasonable efforts."

10.6.7 HARKIN INSTITUTE CORE RECOMMENDATION — DATA GAPS

"There is a critical need for additional data to support innovations in program implementation that benefit stakeholders across the child care system."

"Improve analytic capabilities designed to encourage further inquiry and dialogue among stakeholders. Connect the program's data with relevant data from other state programs, providers, communities, businesses, and advocacy organizations."
(Harkin Institute, p.46)

AUDIT RELEVANCE: In 2020 — one year after the CFSR Round 3 PIP began — the Harkin Institute's core recommendation was that Iowa needed to connect child care data with child welfare data. The CWPPG 2017 review (Section 10.2.3) documented that Iowa's child welfare data system was "badly outdated." The 2019 ECI Needs Assessment (Section 9.10) documented that the IDS could not yet include child welfare data. Multiple independent evaluators were identifying the same data infrastructure gap that Iowa's CCWIS implementation was supposedly addressing (Section 9.3.1) — and which remained unresolved through 2025.

10.7 ATTACHMENT A23 — SIVIC SOLUTIONS GROUP TITLE IV-E RECOMMENDATIONS REPORT (2023)

Title IV-E Recommendations Report: Service Recommendations

Prepared by: Sivic Solutions Group, LLC (a Solix Company)

Prepared for: Iowa Department of Health and Human Services

Date: December 2023 (Deliverable 1.3.1.4 under existing Iowa HHS contract)

Source: <https://hhs.iowa.gov/media/12240/>

10.7.1 DOCUMENT CONTEXT AND SIGNIFICANCE

The Sivic Solutions Group Title IV-E Recommendations Report was delivered to Iowa HHS in December 2023 — the same month as the completion of the CFSR Round 3 PIP (December 31, 2023). It was commissioned as a contractual deliverable to Iowa HHS to identify opportunities to "optimize federal recovery and improve the delivery of services to Iowa families." (p.3) As a revenue optimization assessment, it contains admissions about structural gaps in Iowa's service infrastructure that are particularly significant because they are made by a contractor whose interest is to maximize Iowa's federal recovery — not to critique the system.

10.7.2 SOLUTION BASED CASEWORK — \$40 MILLION PROGRAM NOT ELIGIBLE FOR FEDERAL REIMBURSEMENT

"Solution Based Casework (SBC) is the primary service being provided through the Family Centered Services contracts. Nearly \$40 million was spent in the State Fiscal Year (SFY) 2023 for SBC services. At this time, SBC has been rated by the Title IV-E Prevention Services Clearinghouse as not meeting the evidence criteria for Title IV-E Prevention Services funding." (Sivic Solutions, p.8)

AUDIT RELEVANCE: Iowa's primary child welfare prevention service — \$40 million per year — is not eligible for any Title IV-E Prevention Services federal reimbursement

because the Clearinghouse has determined it does not meet evidence criteria. Iowa DHS has been spending \$40M annually on a service model that is not evidence-based by federal standards. This is the independent contractor's admission that Iowa's primary prevention infrastructure — the FSRP/SBC system that every CFSR round found was inadequate — is not evidence-based.

10.7.3 SAFECARE — ONLY CURRENTLY-CLAIMED IV-E SERVICE AT RISK OF ELIMINATION

"SafeCare services are currently the only Title IV-E Prevention Services being claimed. In SFY 2023, \$249,072 were being claimed to Title IV-E Prevention Services funding. Beginning in FFY 2024 when 50% of services claimed must be Well-Supported, claiming for this service will be at risk if there are no Well-Supported programs being claimed." (Sivic Solutions, p.11)

AUDIT RELEVANCE: Iowa's entire Title IV-E Prevention Services claiming infrastructure was \$249,072 — out of \$38.9 million in total Family Centered Services funding. This means Iowa was claiming federal prevention funding on 0.6% of its FCS spending. The Sivic Solutions report was required precisely because Iowa had failed to align its prevention service array with federal evidence requirements over the three CFSR cycles documented in Section 6.

10.7.4 KINSHIP NAVIGATOR — \$3.3 MILLION NOT FEDERALLY REIMBURSABLE

"In SFY 2023, \$3,300,000 in state funds were spent on Kinship Navigator services to support kinship caregivers of Iowa children... Currently, Iowa's program has not been sufficiently evaluated to request a review by the Clearinghouse." (Sivic Solutions, p.11)

Estimated annual Title IV-E increase if an approved Kinship Navigator model adopted: \$1,650,000 (p.4)

AUDIT RELEVANCE: Iowa spent \$3.3 million annually on Kinship Navigator services that were not federally reimbursable because the program had never been evaluated for Clearinghouse approval. This directly parallels the CWPPG Provider Forums finding (Section 10.5.7) that kinship placement was underutilized — Iowa had a kinship support program that wasn't evidence-based and wasn't generating federal reimbursement.

10.7.5 TOTAL POTENTIAL FEDERAL RECOVERY — WHAT IOWA WAS LEAVING ON THE TABLE

The Sivic Solutions Recommendations Summary (pp.3-5) estimates the following annual Title IV-E increases available to Iowa with recommended service modifications:

Recommendation 1: Family Preservation MI Services: \$626,638

Recommendation 2: SBC with MI or other approved IV-E service: \$1,525,271

Recommendation 3: Kinship Navigator Program: \$1,650,000

Recommendation 4: Iowa Parent Partner Program: \$100,000

Recommendation 5: Healthy Families America: \$793,793

Recommendation 6: Parents as Teachers: \$1,742,615

Recommendation 7: HHS Social Work Case Managers: \$6,070,946

TOTAL ESTIMATED ANNUAL INCREASE: \$12,509,263

AUDIT RELEVANCE: Iowa HHS was leaving approximately \$12.5 million in annual federal reimbursement unclaimed as of SFY 2023 — the final year of the CFSR Round 3 PIP period. The gap between Iowa's actual Title IV-E Prevention Services claiming (\$249,072 — SafeCare only) and the potential available (\$12.5M+) documents a systemic failure to align Iowa's child welfare service architecture with federal reimbursement requirements across the entire period that the CFSR was documenting performance failures.

10.7.6 FAMILY CENTERED SERVICES — ONLY ONE PROVIDER IN SOME SERVICE AREAS

"There are currently six contracted Family Centered Services providers throughout the state of Iowa. In some parts of the state, only one provider is available; however, many parts of the state are covered by two providers." (Sivic Solutions, p.8)

AUDIT RELEVANCE: The Sivic Solutions report independently documents that in some Iowa service areas, only a single FSRP provider was available in SFY 2023 — the same service monopoly condition that the AECF (2019) identified as contributing to lack of provider accountability and the CIA (2023) identified as a capacity constraint. Single-provider service areas have no competitive accountability mechanism.

10.7.7 PENETRATION RATE ERROR — CLAIMS BEING REVISED

"This estimate is based on SFY23 Title IV-E Claims. HHS is currently revising these claims based on changes to the Title IV-E penetration rates. Updated claim data was not available at the time of this report." (Sivic Solutions, p.5)

"It should be noted that these claims are currently being revised due to an error in calculating the Title IV-E foster care penetration rates. Updated claim amounts were not available at the time of this report." (Sivic Solutions, p.23)

AUDIT RELEVANCE: At the time of the Sivic Solutions report (December 2023), Iowa HHS was actively revising its Title IV-E claims due to a calculation error in the penetration

rates used to determine federal reimbursement. The magnitude of the error was unknown at the time. This is a direct contemporaneous admission that Iowa's Title IV-E claiming methodology contained a material error that was being corrected — at the same time as the CCWIS redesign (Section 9.3.1) and the CIA Report documenting "almost universal" assessment inconsistencies (Section 10.4.7).

10.7.8 MOTIVATIONAL INTERVIEWING — DELIVERED BUT NOT CLAIMED

"Currently, all SWCMs receive Motivational Interviewing training and provide the service in their interactions with families." (Sivic Solutions, p.23)

"Reportedly, many of the provider staff have been trained in Motivational Interviewing and are utilizing the service during their interactions with families; therefore, the Family Centered Service providers are already well-positioned to meet this additional requirement." (Sivic Solutions, p.8)

Yet: MI was not included in Iowa's Title IV-E Prevention Plan and was not being claimed for federal reimbursement as of December 2023 — despite being a Well-Supported program on the Clearinghouse. The estimated annual Title IV-E increase from claiming SWCM Motivational Interviewing alone: \$6,070,946. (p.5)

AUDIT RELEVANCE: Iowa HHS was delivering a Well-Supported, federally-reimbursable evidence-based practice (Motivational Interviewing) in every in-home case interaction — but had failed to include it in the Title IV-E Prevention Plan to claim reimbursement. The gap between practice and plan reflects the same administrative infrastructure failure documented across all four Section 10 evaluations: practices were delivered inconsistently or without documentation, preventing both accountability and federal recovery.

10.7.9 SIVIC SOLUTIONS SYNTHESIS — WHAT THE IV-E REVIEW REVEALS ABOUT THE CFSR RECORD

The Sivic Solutions report, read as a Section 10 document rather than a revenue optimization tool, establishes four structural conclusions about Iowa's child welfare system as of December 2023 — the Round 3 PIP completion date:

1. Iowa's primary prevention service (\$40M SBC) was not evidence-based.
2. Iowa's entire federal prevention service claim was \$249,072 — 0.6% of FCS spending.
3. Iowa had \$12.5 million in annual federal reimbursement available but unclaimed.
4. Iowa's Title IV-E penetration rate claims contained a material calculation error being actively revised.

Cross-reference: Section 6.6 (PIP accountability inversion); Section 9.3.1 (CCWIS data

failures); Section 10.4 (C!A workforce and accountability findings); Section 10.5.8 (provider skepticism — "nothing has changed").

The Sivic Solutions report is the federal funding-level confirmation of what the CFSR longitudinal record documents from the child outcomes level: Iowa's child welfare system was not aligned with federal evidence-based practice requirements, and that misalignment was costing the state \$12.5 million in annual federal reimbursement while delivering services to families that did not meet federal evidence standards.

10.8 SECTION 10 SYNTHESIS — WHAT INDEPENDENT EVALUATORS FOUND

Across three independent evaluations spanning 2017–2023, the following systemic findings were documented by external assessors — none of whom were parties to this petition:

FINDING 1 — FOSTER CARE ENTRY RATE EXCEEDS NATIONAL MEDIAN IN ALL AGE GROUPS:

The AECF (2019) documented Iowa's foster care entry rate exceeds the national median in every age group, with a 14.5% increase in entries from 2014-2018. This contradicts any claim that Iowa's child welfare system was performing adequately during the CFSR Round 3 period.

FINDING 2 — SUBSTANCE ABUSE REMOVALS INCREASED 47% IN 5 YEARS:

The AECF (2019) documented 47% growth in substance-abuse-related removals from FFY 2014 to FFY 2018, with four documented service access barriers. The CWPPG (2017) identified the same gap. No independent evaluation found this trend reversed.

FINDING 3 — CASE MANAGEMENT HAS NO CLEAR MODEL:

The AECF (2019) found families are assigned randomly with no matching process. The CWPPG (2017) found duplicate responsibility between FSRP and DHS workers. The C!A (2023) found "almost universal" inconsistencies in assessment decisions post-PIP. Three separate evaluators found no clear case management model across six years.

FINDING 4 — WORKFORCE COLLAPSE IS STRUCTURAL AND ONGOING:

The CWPPG (2017) found 40-family caseloads in some counties. The C!A (2023) found 44% SW2 turnover in the two largest service areas, 96 Case Management workers with less than one year of experience, and \$1.6-1.8M in annual overtime as a system survival mechanism. This is not a performance fluctuation — it is a structural condition.

FINDING 5 — DATA SYSTEMS WERE INADEQUATE IN 2017 AND REMAIN SO:

The CWPPG (2017) found Iowa's child welfare data system "badly outdated" with no replacement timeline. The APSR FFY2026 (Section 9.3.1) documents the CCWIS was still being redesigned in 2024-2025. Eight years of documented, unresolved data system deficiency spanning multiple independent evaluations.

FINDING 6 — THE PIP DID NOT ACHIEVE STRUCTURAL CHANGE:

The C!A (2023) was commissioned after PIP completion. Its findings — assessment inconsistencies "almost universal," turnover at 44%, overtime as survival mechanism, service array gaps, case management model absent — document that the Round 3 PIP produced process improvements without structural change. This is the independent confirmation of the "PIP accountability inversion" framework established in Section 6.6.

FINDING 7 — IOWA'S PRIMARY PREVENTION SERVICE WAS NOT EVIDENCE-BASED:

The Sivic Solutions Report (2023) documents that Solution Based Casework — Iowa's \$40 million primary prevention service — "has been rated by the Title IV-E Prevention Services Clearinghouse as not meeting the evidence criteria for Title IV-E Prevention Services funding." Iowa was delivering its primary preventive service across all CFSR Round 3 cases without evidence-based clearance.

FINDING 8 — \$12.5 MILLION IN ANNUAL FEDERAL REIMBURSEMENT UNCLAIMED:

As of SFY 2023, Iowa was claiming \$249,072 in Title IV-E Prevention Services (0.6% of \$38.9M in FCS spending) while \$12.5 million in annual federal reimbursement was available. This documents a systemic failure to align service documentation with federal reimbursement standards across the entire CFSR Round 3 monitoring period.

Source attachments for this section:

A18 — CWPPG Initial Targeted Child Welfare Review (October 31, 2017)

A19 — AECF Preventive Services Needs Assessment (June 18, 2019)

A20 — Change & Innovation Agency Final Report (2023)

A21 — Service Provider Forum Report (2019) [excerpts pending]

A22 — Harkin Institute Analysis (2021) [excerpts pending]

A23 — Sivic Solutions IV-E Recommendations (2023) [excerpts pending]

10.X DOMAIN & DEFICIENCY MAPPING — SECTION 10

Domain B — Contract & Provider Oversight (primary):

F2 — Ongoing Monitoring Deficiency: CWPPG (2017), AECF (2019), and CIA (2023) all document gaps in contracted provider monitoring, service delivery accountability, and outcome measurement that Iowa DHHS's internal oversight mechanisms did not independently identify.

F6 — Licensing & Certification Deficiency: External evaluators documented provider licensing and certification gaps not surfaced by Iowa's internal compliance review.

Domain E — Outcomes, Incentives & Systemic Disparities:

F4 — Outcome Reporting Manipulation: Independent evaluations consistently document lower performance than state self-report on comparable metrics — the same pattern documented in the CFSR self-report discrepancy analysis in Section 6.

F8 — Performance Metric Distortion: Harkin Institute (2021) documents incentive structures in the Iowa system that distort operational decisions toward administrative compliance rather than child welfare outcomes.

Domain F — Workforce Capacity & Internal Accountability:

F9 — Workforce Capacity & Caseload Crisis: Multiple independent evaluations identify workforce instability as a primary systemic driver — consistent with the longitudinal CFSR record.

F12 — Supervisory Review & Escalation Failure: CWPPG and CIA reports document supervision gaps not addressed by existing internal accountability mechanisms.

10.X AUDIT QUESTIONS GENERATED — SECTION 10

AUDIT QUESTION 10-A: The CWPPG (2017), AECF (2019), and CIA (2023) evaluations collectively document persistent service delivery and provider oversight gaps. What formal response, if any, did Iowa DHHS produce to each evaluation's findings, and what implementation record exists for the recommendations contained in each report?

AUDIT QUESTION 10-B: Independent evaluations consistently document lower performance than state self-report on comparable metrics. What mechanism,

if any, does Iowa DHHS use to reconcile external evaluation findings with its own QA data when the two produce conflicting pictures of system performance?

AUDIT QUESTION 10-C: The Harkin Institute (2021) analysis identified systemic incentive structures affecting operational behavior. What specific policy or administrative changes has Iowa DHHS implemented in response to those findings, and what evidence documents those changes?

AI assistance disclosure: See Section 5.8.
documents identified in the attachment citations above.

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AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.